A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

29 April 2026

Mr Eugene Ong
Melbourne Colorectal Specialists

Dear Mr Ong,

Re: Mr Dylan Aldous
DOB: 27/07/1983 Phone: 0401 864 598
Address: 39 Beaurepaire Parade, FOOTSCRAY, VIC, 3011
Email: dylanraldous@gmail.com

Summary

- **Solitary rectal ulcer syndrome (SRUS) secondary to mucosal prolapse and pelvic floor dyssynergia.**
- **Commence six-week trial of Predsol suppositories; for consideration of referral to physiotherapist.**
- **No action required by you.**

I had the pleasure of conducting an in-person consultation with Dylan Aldous.

Symptoms and history of presenting complaint

Dylan presents with a primary concern of persistent rectal mucus leakage, which first became prominent in late 2024. He has a significant history of Grade 3 internal haemorrhoids, for which he underwent banding surgery by a general surgeon in late 2024, followed by several rounds of sclerotherapy injections by yourself. While these interventions provided marginal relief, the mucus discharge persists, particularly during physical activity such as football and in warmer weather.

Dylan describes a long-standing history of chronic straining and a sensation of incomplete evacuation. His current bowel habit involves approximately two movements each morning; while the first feels complete, he often feels the urge to return shortly thereafter to pass a smaller, secondary volume. He reports occasional central abdominal pain in the early morning that resolves following defecation. Notably, he denies any recent haematochezia, although this was the original impetus for his colonoscopy, which identified haemorrhoids but was otherwise unremarkable. He has previously engaged with a pelvic floor physiotherapist to address both bowel and urinary urgency symptoms.

Diet

Dylan has recently modified his diet to include high-fibre smoothies containing banana, oats, seeds, and nuts. He has previously trialled psyllium husk and partially hydrolysed guar gum (PHGG) with some success in improving stool consistency. He has significantly reduced his caffeine intake, as he noted a direct correlation between coffee consumption and increased urinary urgency and rectal leakage.

Past medical history and medications Dylan's medical history is notable for obstructive sleep apnoea, for which he uses a CPAP machine with good effect, as well as childhood adenoidectomy and grommet

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

insertions. He has been reviewed by a urologist for bladder neck obstruction and urinary urgency. His current medications are minimal; he has recently ceased fish oil supplements and uses Temazepam approximately once or twice weekly for sleep maintenance during periods of heightened stress.

Social history, family history

Dylan is a trained psychologist currently working in a management role within the public service. He identifies as having an anxious predisposition and notes that his symptoms often fluctuate with psychosocial stressors, such as a recent house move. He lives with his partner and two young children. His family history is significant for a father with similar issues involving haemorrhoids and anxiety.

Examination I performed a physical examination which revealed a soft, non-tender abdomen with no palpable masses or organomegaly.

Impression and plan

The clinical picture is highly suggestive of solitary rectal ulcer syndrome (SRUS). This appears to be driven by a possible mild mucosal prolapse, likely a consequence of long-term pelvic floor dyssynergia and chronic straining.

My management plan is as follows:

- **Predsol suppositories:** I have prescribed a six-week course of prednisolone suppositories to be used nocturnally. This aims to reduce localised mucosal inflammation and diminish mucus production.
- **Physiotherapy:** While Dylan has previously undertaken pelvic floor exercises, I have discussed the potential benefit of seeing a physiotherapist specialising specifically in colorectal biofeedback and rectal balloon training to further address the underlying dyssynergia. We have decided to put this on hold depending on his response.
- **Neuromodulators:** We discussed the optional use of low-dose Amitriptyline to assist with bowel frequency and secretions; however, Dylan prefers to trial topical therapy first.
- **Review:** I will review Dylan in eight weeks to assess his clinical response to the Predsol trial.

Thank you again for referring this patient.

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist

CC: Dr Erwin Wong

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065